Optimizing the Nutrition Care Pathway

Product Presentation

Jan 2024

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Transforming the management of clinical nutrition

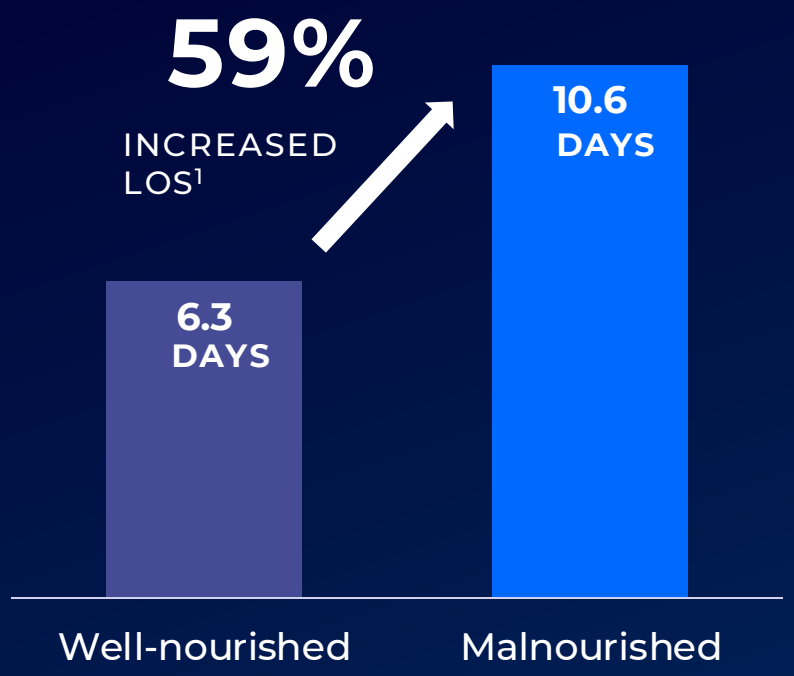
Dim3 is establishing a data continuum between hospitals, nursing homes and homecare so that nutrition quality of care is optimized throughout the **entire patient journey**



SECTION 01

Unmet Clinical Need

Malnutrition is Costly



UNTREATED MALNUTRITION COSTS

€170B

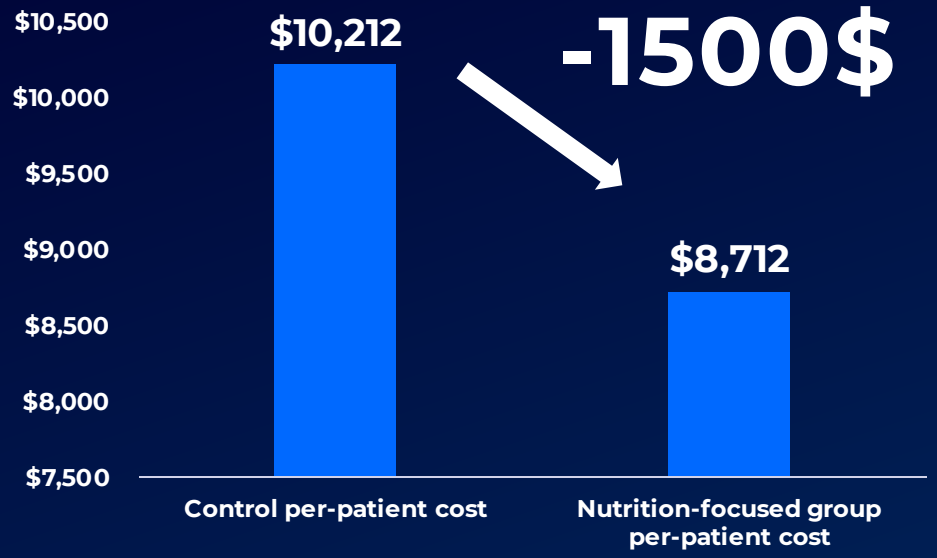
per year in EU[2]

INCREASED LENGTH OF STAY (LOS)

INCREASED HEALTHCARE COSTS

1. https://clinicalnutritionespen.com/article/S2405-4577(15)00097-2/pdf
2. https://www.nature.com/articles/ejcn2014280

Acting on malnutrition saves money!



TREATING MALNUTRITION AT-HOME WOULD SAVE

€1,7B

per year in the US[1,2]

NUTRITION-FOCUSED PROGRAM SAVES MONEY

POTENTIAL COST SAVINGS

1. https://onlinelibrary.wiley.com/doi/10.1002/jpen.1606
2. https://www.cdc.gov/nchs/fastats/home-health-care.htm

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Root Causes of Malnutrition

Poor Detection

Patients at malnutrition risk are not detected or not detected soon enough

Lack of Knowledge

Professionals lack knowledge and sometimes interest in nutrition, resulting in non-optimal practices



Less than 60% Compliance[1,2]

Patients don't receive what they have been prescribed. Very few know the issue

Multi-step, Disconnected Care

Multiple in-hospital transitions, lack of coordination between hospital and home and the trend toward shorter LOS intensifies an issue that is already ill-addressed

1. https://pubmed.ncbi.nlm.nih.gov/19572118/
2. https://pubmed.ncbi.nlm.nih.gov/31684024/

SECTION 02

Clinical Solution

Individualized Support Across the Continuum

Dim3 is establishing a data continuum between hospital, nursing homes and homecare to improve patient outcomes

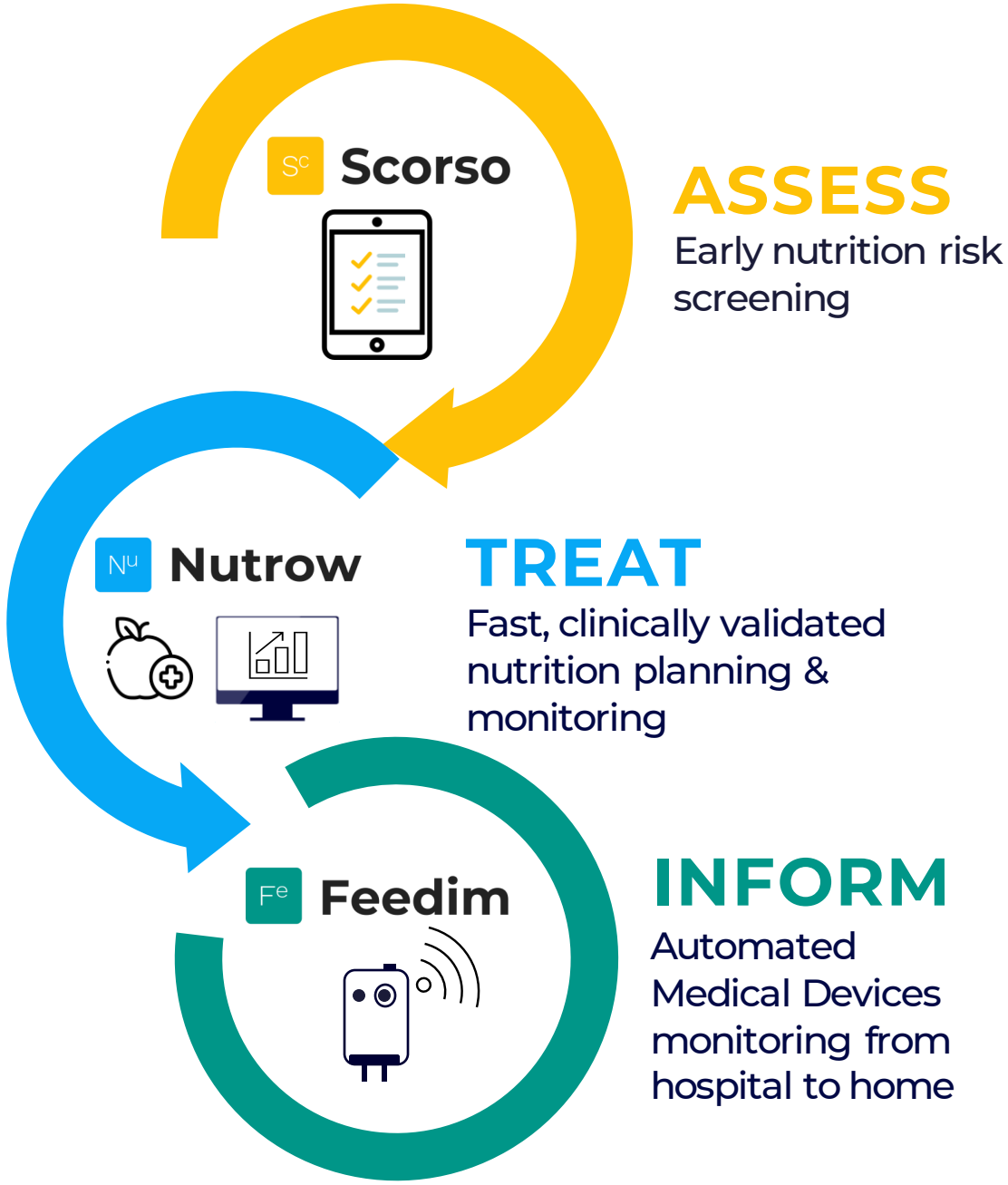


The First 360° Clinical Nutrition Management System

FROM HOSPITAL→ HOME



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Proactive Nutritional Screening

Scorso

- Easy, fast and reliable nutrition risk assessment
- Scientifically validated scoring procedures*
- Direct integration with Nutrow



Assess nutritional status at any point during care journey



Built-in patient history & customizable alarms



"Smart" integration to external and mobile devices

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* MNA, NRS2002, GLIM coming soon

360° View of Nutrition Care Pathway

Nutrow

- Improve patient compliance
- Deliver actionable nutrition insights
- Reduce healthcare costs
- Zero double data entry

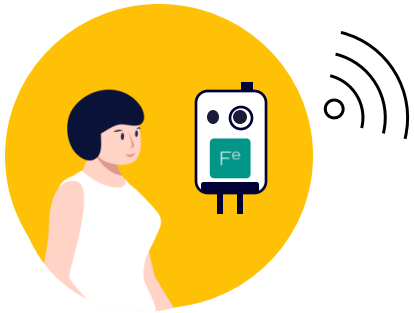
CE



Gather all nutrition related data in actionable dashboards for earlier prevention and tailored nutrition planning



Streamlines communication and optimizes patient care across the care continuum



Nutrow integrates real-time pump intelligence for faster, informed clinical decision making

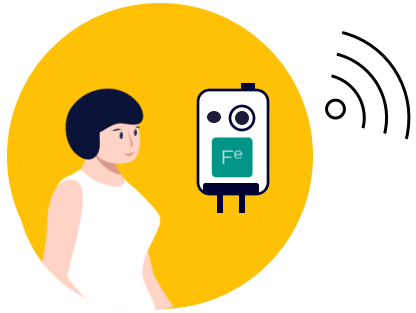
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Real-Time Pump Intelligence

Feedim

- Capture feeding pump data without effort
- Save time
- Unlock data for a better clinical nutrition follow-up
- Eliminate data entry mistakes

CE



Real-time, remote insights into nutrition levels for targeted, proactive support



Rapid pump intelligence expedites treatment adjustments and automates burdensome data collection



Manage large pump fleets, proactively troubleshoot and easily trace devices

Supports leading enteral pumps : Nestle, Danone-Nutricia, Fresenius Kabi, Cardinal Health

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The Virtuous Cycle of Better Nutrition

KPI from Scientific Literature and Society Recommendations

BETTER COMPLIANCE

91%

RX calories delivered to patients with computer-based decision support system[1]

SAVE TIME

68%

Less work time per patient with a nutrition management system[2]

REDUCE COSTS

€3.300+

Cost savings per malnourished patient on nutrition-focused program[3]

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ESPEN European Society for Clinical Nutrition and Metabolism

aspen American Society for Parenteral and Enteral Nutrition

eat right. Academy of Nutrition and Dietetics

acdis

1. https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3667982/
2. https://www.ncbi.nlm.nih.gov/pmc/articles/PMC3002126/
3. https://pubmed.ncbi.nlm.nih.gov/28975010/

Benefits



Patients

- Optimize Tx compliance
- Early prevention
- Individualized care
- Designed to improve outcomes:
  - ↑Survival
  - ↓ LOS
  - ↓ Readmissions
  - ↓ Admission to ICU



Nutritional Team

- Optimize workflow & team communication
- Centralize patient nutritional status
- Reduce errors associated with manual data entry
- Simulate optimal nutrition
- Adopt clinical guidelines
- Support scientific research with structured database



Hospital

- Evidence-based, 1st-class support
- Recognized cost savings
- Additional revenues by optimally documenting malnutrition
- Stimulate interdisciplinary/ inter-hospital dialogue
- Concrete building block for JCI/AC or other professional accreditation programs



Homecare Provider

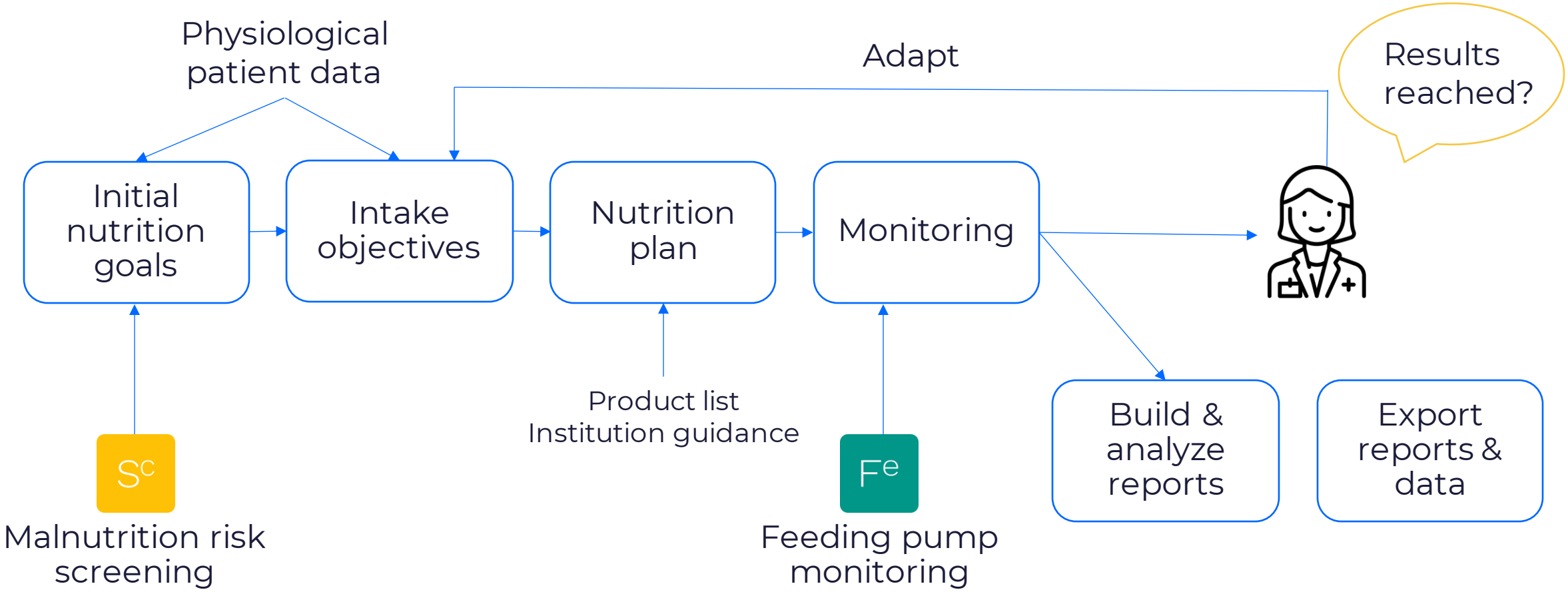
- Better service and support
- Service differentiation
- Maximize compliance
- Accuracy of nutrition delivery
- Remote technical assistance
- Reduce unnecessary visits
- Efficient stock management

SECTION 03

Manage the Full Nutrition Workflow

Manage the full Nutrition Worklow

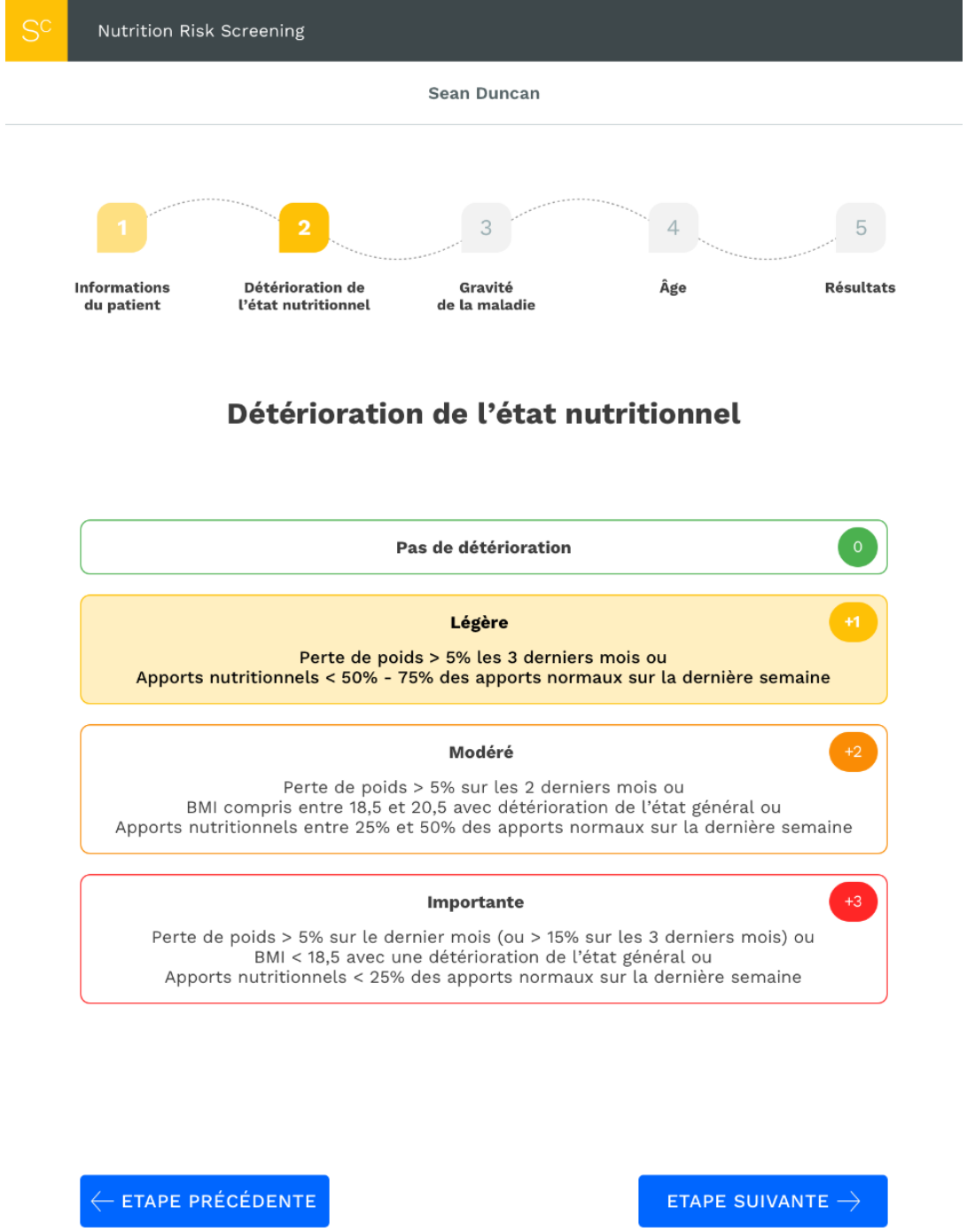
Nutrow



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Scorso

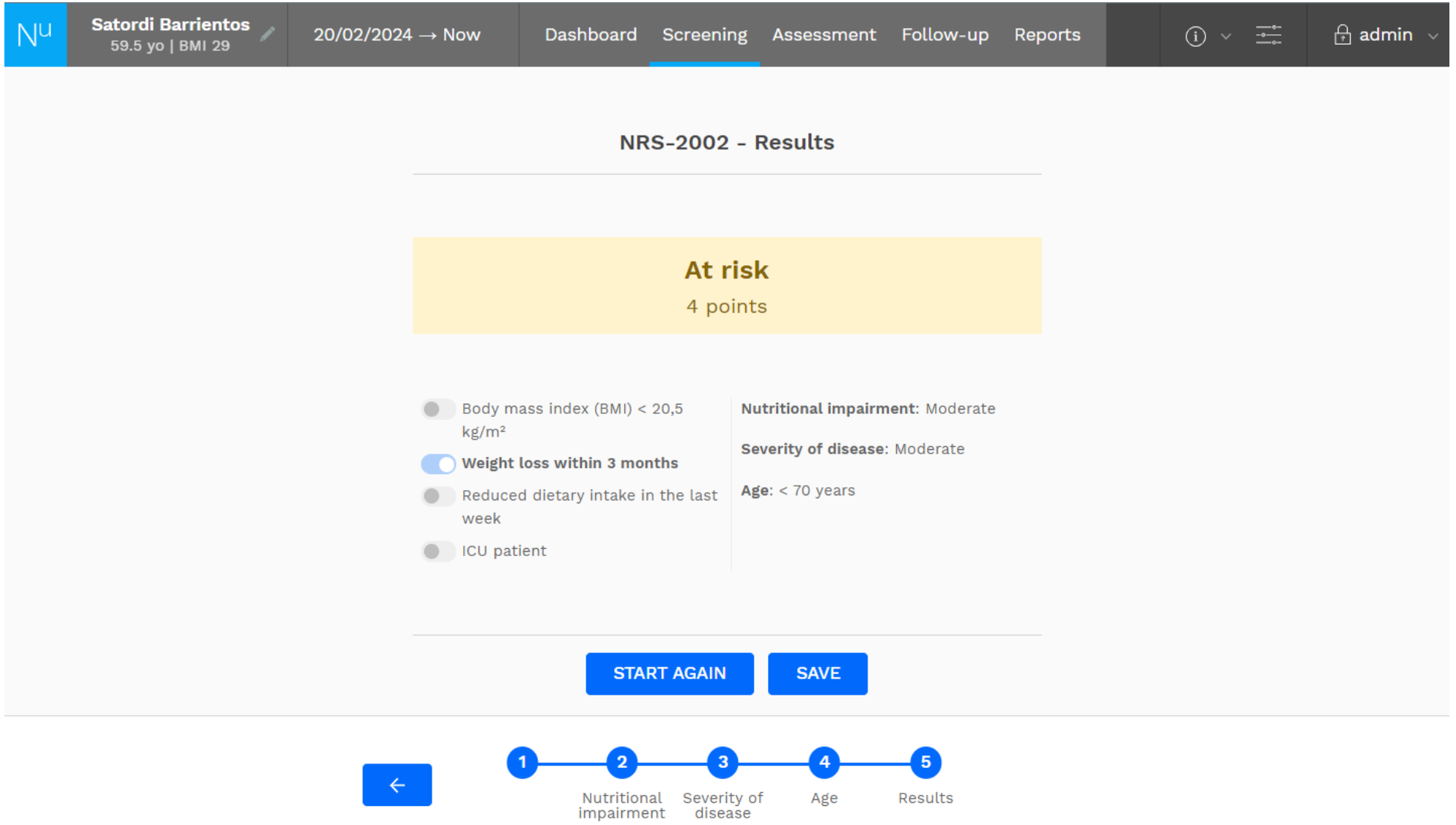
Proactive Nutritional Screening



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Screen and Assess in Nutrow too

Nutrow



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Define Goals

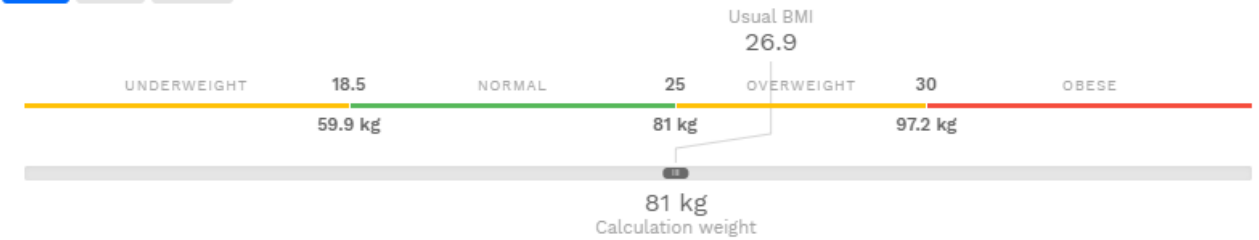
Nutrow

Measures

Height: 180 cm

Weight: 87 kg

Default | Lorentz | Robinson



Patient is overweight. **Suggestion:** Use the weight at a BMI of 25 for BMR calculation.

BMR formula

Patient has burns

Basal metabolic rate formula: Toronto

More info on formulas

Burn date: 05/12/2018

Burn area: 50 %

Kcal intake in the last 24h: 0 kcal

Temperature: °C

Initial targets

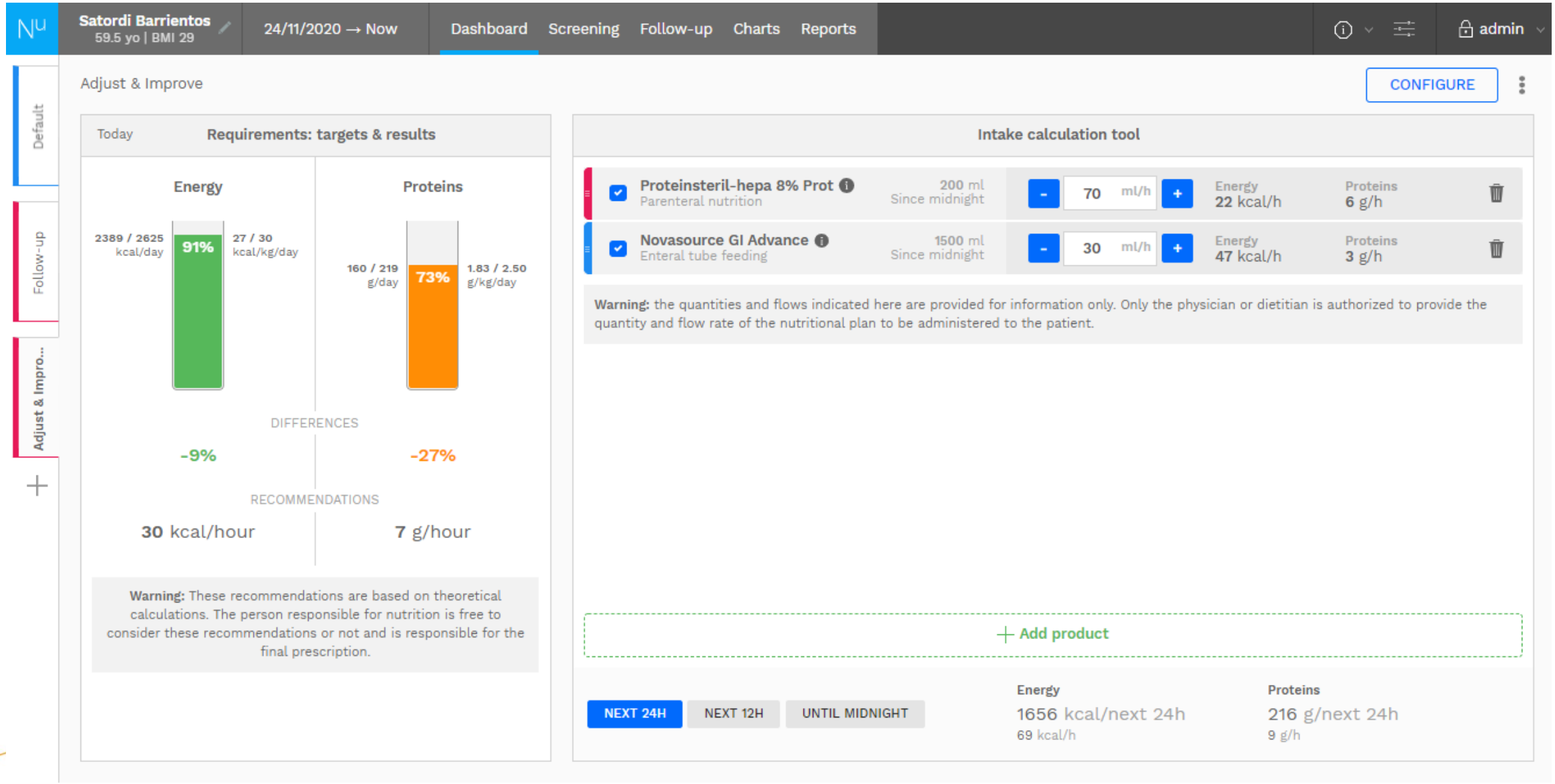
	Energy targets	kcal	kcal/kg
○	Toronto	n/a	n/a
○	20 kcal x calc. weight	1620	20
●	**25 kcal x calc. weight**	**2025**	**25**
○	30 kcal x calc. weight	2430	30
○	35 kcal x calc. weight	2835	35
○	kcal/kg		

	Protein targets	g	g/kg
○	1 g x calc. weight	81	1
○	1.25 g x calc. weight	101	1.25
○	1.5 g x calc. weight	122	1.5
○	1.75 g x calc. weight	142	1.75
●	**2 g x calc. weight**	**162**	**2**
○	g/kg		

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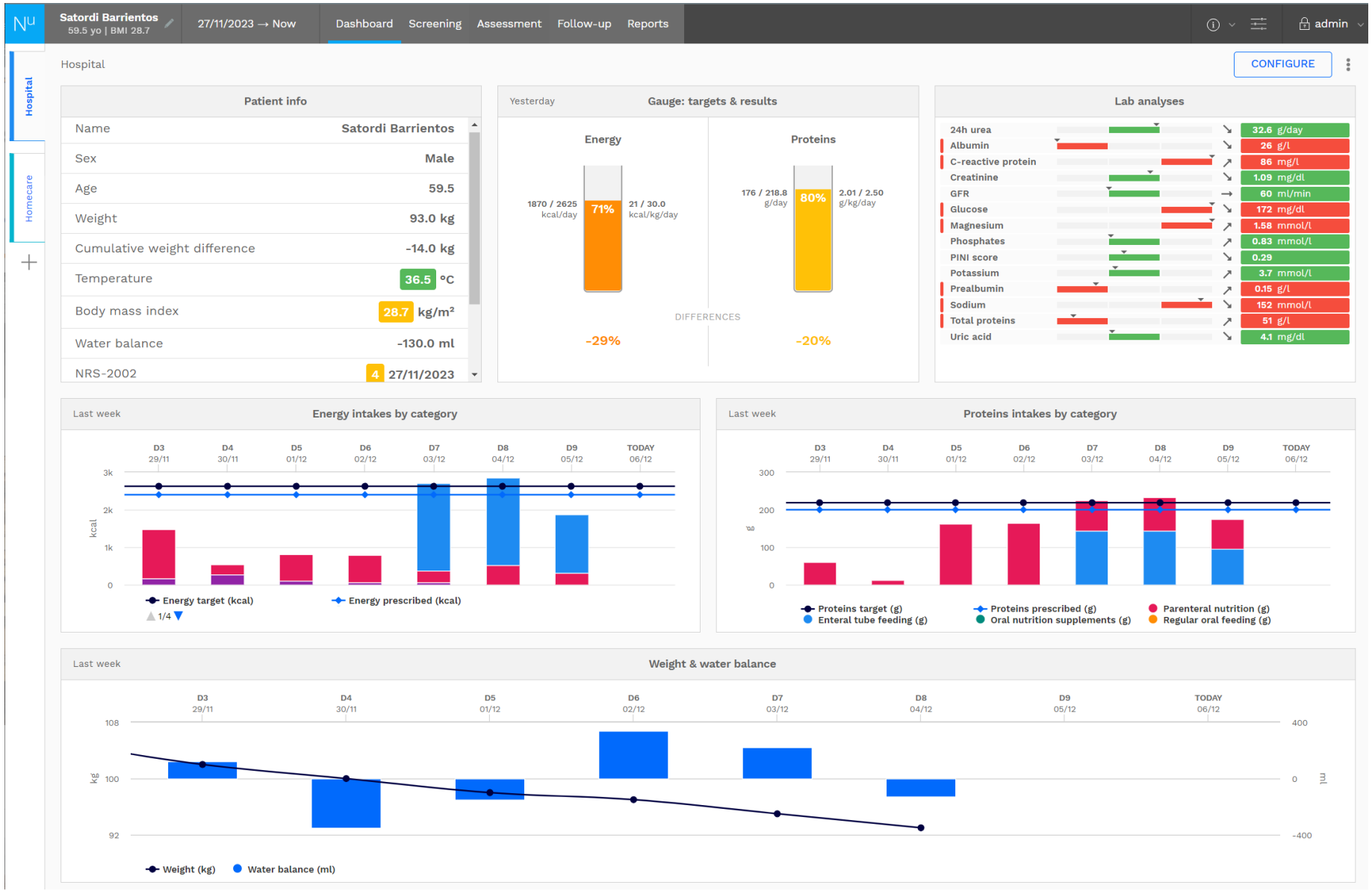
Set - Improve Nutrition Plan

Nutrow



Follow-up

Nutrow



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Real-Time Pump Intelligence

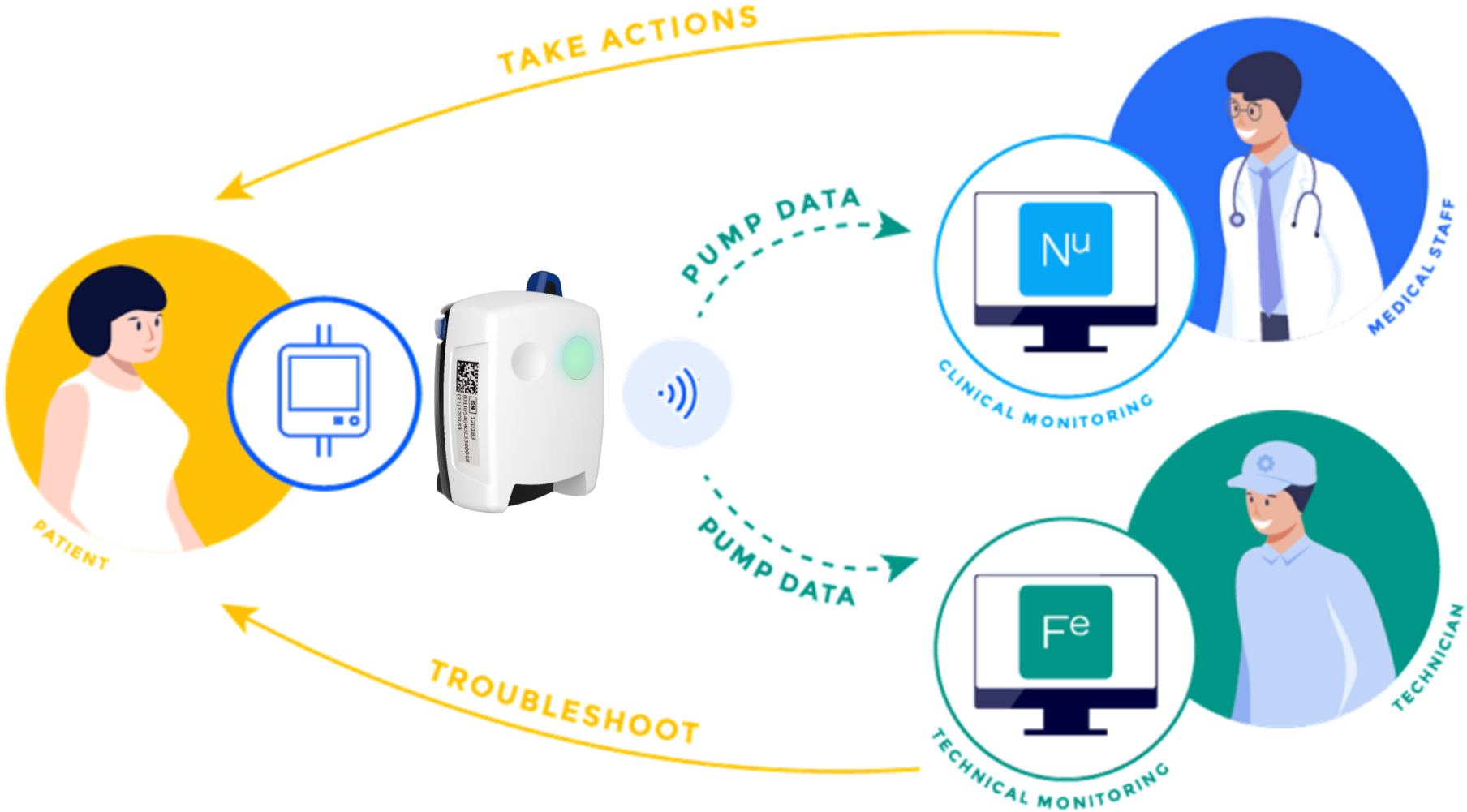
Feedim



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Capture Feeding Pumps Data Without Effort

Feedim



Capture Feeding Pumps Data Without Effort

Feedim

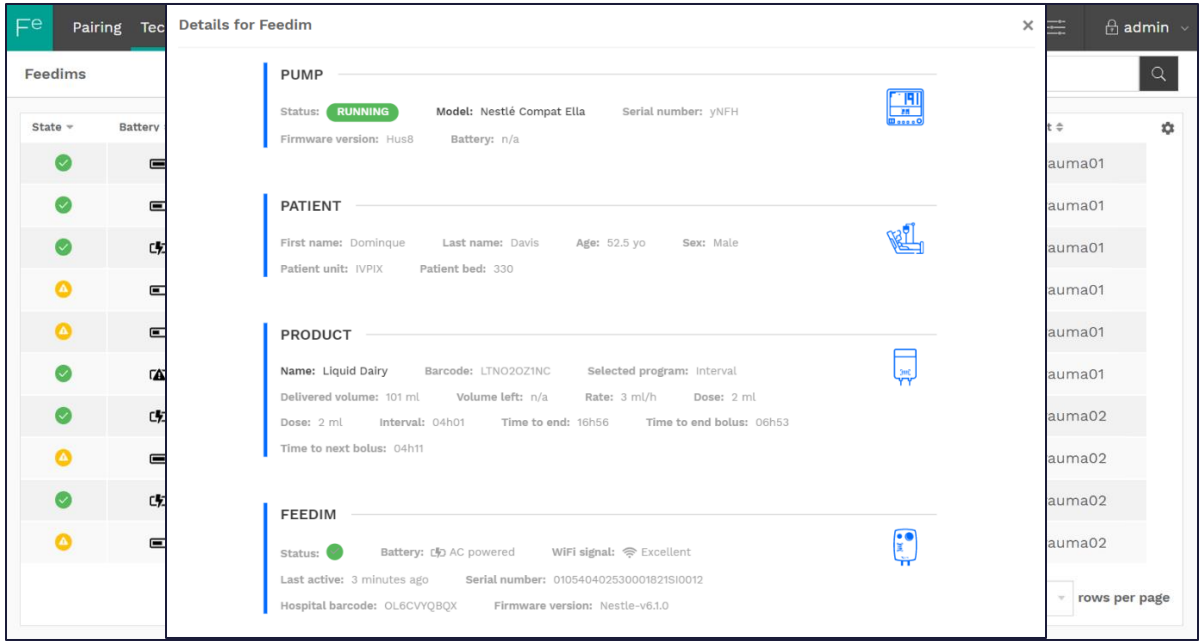
Feedim Device



Feedim cable and Pump cable



Feedim Software



Already compatible with :









First Parenteral Pump

Dim3

THANK YOU

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SECTION 06

Testimonials

“Without exact data we are unable to make further progress. The smart platforms and IT solutions developed by Dim3 are welcome technical solutions to enable personalized feeding, facilitate monitoring and potentially change the outcome of our patients.”

PROF. DR. ARTHUR (A.R.H.) VAN ZANTEN, MD, PhD
Wageningen University and Research, Division Human Nutrition and Health
Internist-intensivist Gelderse Vallei Hospital in Ede, The Netherlands
ESPEN guideline member Nutrition for the adult critically ill patient
NESPEN Executive Board, Associate Editor Clinical Nutrition
Member of Dim3 Scientific Committee

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“Appropriate nutritional support is a key component of care for critically ill patients. While malnutrition increases complications, impacting long term outcomes and healthcare-related costs, uncertainties persist regarding optimal provision of nutritional support and how to monitor this in this setting.

As a Steering Committee member of the German national nutrition guidelines (DEGEM and ESPEN education member) and an active nutrition specialist and researcher in this field (>150 publication), I consider Dim3 and its novel technology as promising strategy to provide first time a reliable tool to monitor real-time nutrition practice on ICU, with the overall potential to significantly improve patients outcomes worldwide.”

PROF. DR. C. STOPPE, FAHA
Department of Anesthesiology
Intensive Care Medicine and Pain Therapy
University Hospital Würzburg, Germany
3CARE - cardiovascular critical care & anesthesia evaluation and research
Member of Dim3 Scientific Committee

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“Nutrow helps me taking better care of brain damaged patients with artificial feeding.”

PROF. DR. STEVEN LAUREYS, MS, MD, PhD, FEAN

Director of Brain2 Clinic, University Hospital of Liège & GIGA Consciousness Research Unit – Coma Science Group, University of Liège, Belgium
Co-director of the Hangzhou International Coma & Consciousness Institute
Founder & Past Chair of the World Federation of Neurology Applied Research Group on Coma & the European Academy of Neurology Scientific Panel on Disorders of Consciousness
Member of the Royal Academy of Medicine of Belgium
Member of Dim3 Scientific Committee

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“It is worldwide admitted that malnutrition is a risk factor that may alter the outcome and the quality of life of patients. During the last decade, the importance of medical nutrition has been growing and must be totally integrated in the global medical strategy. Nutrow and Feedim help optimize the administration and the monitoring of medical nutrition. The use of such system in the hospital but also in homecare is of major value. This contributes to reach a “patient-driven personalized medicine”, that is the future in medicine.”

ANDRE VAN GOSSUM, PhD

Gastroenterologist
Past ESPEN chairman
Professor at the Free University of Brussels, Belgium
Consultant in the department of gastroenterology/nutritional support at Erasme University Hospital and Bordet Institute
Member of Dim3 Scientific Committee

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“It Medical nutrition is so important and so often overlooked in daily practice that easy-to-use and customizable tools designed to assist the prescription and delivery of nutrition formulas likely provide a great help to every healthcare professional.”

JEAN-CHARLES PREISER, MD, PhD

Internist-intensivist
Professor of Nutrition, Faculty of medicine and biomedical science Free University of Brussels, Belgium
Medical director for Biomedical Research and Clinical Teaching, Erasme University Hospital, Brussels
Past Chairman of the Belgian Society of Clinical Nutrition
Past-vice chairman of the French-speaking society of clinical nutrition and metabolism
Member of Dim3 Scientific Committee

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“Dim3 understands the needs of doctors, dietitians and nurses, and the challenges of building comprehensive data workflows in a big modern hospital.
They came up with easy-to-use and directly actionable user interfaces, and a state-of-the-art, open approach to data management.
They listened to our requirements and integrated smoothly with our EMR. They are quick to react on any request users or my IT department have.
That's the future of IT in healthcare.”

MICHEL RAZE, MSc

Head of IT Department, University Hospital of Liège, Belgium
Senior Lecturer, University of Liège

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“As a nurse, it's a blessing when technology allows us to do less administrative work and more clinical work.
Feedim does just that, creating monitoring of nutrition delivery that simply didn't exist before in some places or improving it in others, making it almost real-time so that we can instantly act upon deviances to objectives and help our patients.
This saves data entry time and protects us from inevitable manual data entry errors while helping us ensure a better quality of care.
Dim3 cares about our practice and integrates our feedback into the product. It's a win-win and we’d hope we could use Feedim with other medical devices in the future.”

Olivier THONON, RN, CCRN, MPH, PhD

Senior Head Nurse, Nursing Department, University Hospital of Liège, Belgium

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“The gathering of all nutritional data is undoubtedly essential in order to complete the patient care and follow-up in the medical record of the hospital. In this context, our collaboration with Dim3 has proved fruitful. I am thinking in particular of the implementation of Nutrow which allowed us to federate the IT expertise of Dim3 and the business expertise of the CHU on a common project.

CHU Liège also had the opportunity to collaborate with Dim3 on the analysis of "workflow" aspects for their Feedim by allowing them to test the tool in an intensive care unit. We are indeed sensitive to projects of this type, the objective of which is to develop tools to eliminate manual copying of data, which generates errors and overloads for the nursing staff.”

PROF. PHILIPPE KOHL, MD, PhD

Professor of Human Biochemistry and Physiology, University of Liège , Belgium
Chief Information Officer, University Hospital of Liège
Editor-in-Chief Emeritus, European Journal of Vascular and Endovascular Surgery
Deputy President Elect, European Society for Vascular Surgery

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